

Clinical Operations Manual

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Last Updated: April 2025

1. Triage Protocol

All patients presenting to the Emergency Department must be triaged within 15 minutes of arrival. The Emergency Severity Index (ESI) five-level system is used:

Level	Name	Time Target	Examples
1	Resuscitation	Immediate	Cardiac arrest, seizure
2	Emergent	10 minutes	Chest pain, stroke symptoms
3	Urgent	30 minutes	Moderate pain, lacerations
4	Less Urgent	60 minutes	Simple fracture, UTI
5	Non-Urgent	120 minutes	Rx refill, minor rash

The triage nurse must document chief complaint, vital signs, pain assessment, allergies, and current medications. Any patient with chest pain must receive a 12-lead ECG within 10 minutes of triage.

2. Admission and Bed Assignment Policy

Bed assignments are managed by the Bed Management Office (BMO), operational 24/7. Assignments are based on clinical need, bed availability, and isolation requirements.

Priority	Bed Type	Criteria
1	ICU	Continuous monitoring, mechanical ventilation
2	Telemetry	Continuous cardiac monitoring
3	Med/Surg	Standard inpatient by department
4	Isolation	Known/suspected infectious disease

Capacity Escalation

When occupancy exceeds 85%, the BMO activates the Capacity Escalation Protocol: expedite discharges, open overflow areas, notify CMO. Patients may not be held in ED for more than 4 hours awaiting bed assignment.

3. Discharge Planning Procedures

Discharge planning begins at the time of admission. The interdisciplinary care team is responsible for developing and executing the discharge plan.

Step	Action	Responsible Party
1	Assess discharge needs within 24 hrs	Case Manager
2	Disease-specific patient education	Physician / Nursing
3	Medication reconciliation	Pharmacist

4	Schedule outpatient follow-up	Case Manager
5	Transportation arrangements	Social Work

High Readmission Risk (Score > 0.7)

Enhanced discharge: post-discharge call within 48 hours, home health referral, primary care appointment within 3 days.

4. Readmission Prevention Program

Our hospital participates in the CMS Hospital Readmissions Reduction Program. Target conditions: heart failure, pneumonia, COPD, hip/knee arthroplasty, and CABG.

- Risk Stratification: All admitted patients receive a readmission risk score. Patients scoring > 0.7 are flagged as high-risk.
- Teach-Back Method: Nursing staff ensure patients understand discharge instructions, medications, and warning signs.
- Transition Care Nurses: Dedicated nurses follow high-risk patients for 30 days post-discharge (calls at days 2, 7, 14, 30).
- Medication Adherence: Pharmacy provides pre-filled pill organizers and counseling for patients on 5+ medications.
- Community Partnerships: Referrals to community health workers for patients with social determinants barriers.

5. Quality and Safety Standards

All clinical departments are required to track and report the following quality metrics monthly:

Metric	Target	Frequency
Hospital-Acquired Infections	< National Benchmark	Monthly
Patient Falls	< 3.0 per 1,000 days	Monthly
Medication Error Rate	< 0.5%	Monthly
30-day Readmission Rate	< 12%	Monthly
Avg LOS vs Expected LOS	Ratio < 1.1	Monthly
HCAHPS Satisfaction	> 75th percentile	Quarterly
Mortality Rate by DRG	< Expected	Monthly

Critical Lab Values (notify provider within 30 min)

Troponin > 0.04 ng/mL | Potassium > 6.0 or < 2.5 mEq/L | Glucose > 500 or < 40 mg/dL | Lactate > 4.0 mmol/L | Positive blood culture

All adverse events must be reported through the Patient Safety Reporting System within 24 hours. Root cause analysis is required for sentinel events.